

Our Ref: (T) // [MED-ID]

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division C

[ADDRESS]

Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] , [ADDRESS] [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Problem: Panenteric Crohn's disease (diagnosed February 2014).

Initial Treatment: [NAME] IBD as a complete feed for eight weeks followed by controlled food introduction.

Investigations (February 2014):

Gastroscopy - gastritis seen endoscopically with oesophagitis, gastritis and duodenitis on hi

Colonoscopy - active ileitis, confirmed histologically with patchy colonic inflammation confi
histology

New Management:

Restart [NAME] IBD as a complete feed for four weeks then controlled food re-introduction

Continue multivitamin and mineral supplement

Consideration of azathioprine (not discussed yet)

I reviewed Binta in clinic on 15 July, somewhat earlier than planned at mum's request.

She has had a flare-up of her Crohn's disease with principally abdominal pain. [NAME] has re
reduced intake and weight loss and her weight now been down at 28.4kg (30.05 kg April 2014).

She feels [NAME] was very helpful at time and is keen to restart. I have therefore suggested
course in the first instance.

I have checked her bloods. I will request dietetic review. She has a planned appointment wi

[PROVIDER-NAME] next week which I have suggested that she keeps.

It may be longer term she would benefit from the introduction of azathioprine.

I have attached a copy of her blood results.

Yours sincerely

[PROVIDER-NAME]

Consultant Paediatric Gastroenterologist

Copies:

[ADDRESS]

[ADDRESS]

[ADDRESS]

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[POSTCODE]

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